

KMC Speciality Hospitals (India) Ltd

Concall-Tracker report · Kauvery Hospital, Trichy · BSE 524520 / KMCSHIL · continuity update: Q1 FY27 newly read · no concall transcripts — web deep-dive · prepared 31 Aug 2026

1. Snapshot

What the business does (one line): KMC runs hospitals in **Trichy (Tiruchirapalli), Tamil Nadu** under the **Kauvery Hospital** brand — treating in-patients and out-patients across mother-and-child care, neurology, gastroenterology, orthopaedics, critical care and transplants. It operates a 250-bed legacy multi-speciality block and a 200-bed mother-and-child hospital opened in January 2024. It is **75%-owned by Sri Kauvery Medical Care**, a 12-hospital South India group, and is that group's only listed company.

Figure	Value	Plain-English read
Market capitalisation	₹2,209 cr (price ₹136)	What the whole company is worth. The share has ranged ₹65–148 over the past year — it has more than doubled off the low.
Price-to-earnings	39.6x; no dividend	You pay about ₹40 for every ₹1 of annual profit. Expensive — though notably <i>cheaper</i> than the 47x of three months ago, because profits grew faster than the price.
Q1 FY27 revenue (Jun'26 qtr)	₹91.8 cr, up 37.9% YoY	Revenue from patients in the June 2026 quarter, against ₹66.6 cr a year earlier — the best quarter in the company's history.
Q1 FY27 operating profit	₹28 cr, margin 31%	About 31 paise of operating profit on every rupee of revenue, up from 25% a year earlier. A strong hospital margin, and still widening.
Q1 FY27 net profit	₹16.6 cr, up 119.9% YoY	Profit after tax more than doubled; pre-tax profit ₹22.4 cr, up 124.2%. Earnings per share ₹1.02 for the quarter.
Occupancy	82% overall; legacy block ~98%	How full the beds are. The original multi-speciality hospital is now effectively full again — which is exactly what has triggered the new expansion described below.
Revenue per occupied bed (ARPOB)	~₹31,481/day, up 17% YoY	The average daily revenue earned from each occupied bed. A 17% rise is well above the mid-single-digit increases of prior years — a richer mix of complex cases.
FY26 (prior full year)	₹306 cr revenue, ₹47 cr profit	Revenue up 32% and profit up 118% — the year the new block finally paid off.
Return on capital	ROCE 26.0%; ROE 24.9%	For every ₹100 of capital the business earns about ₹26 a year before tax — strong for a hospital.
New expansion approved	₹519 cr, ~591 beds	Board-approved on 14 August 2026. This roughly doubles the bed base and is the single biggest development since the last report.

2. Concall coverage

This is a **continuity update**. As before, **no concall transcripts are available on Screener for this company — this report is based on a global web deep-dive**. KMC holds quarterly earnings calls and publishes an investor presentation with each, but the transcript PDFs are login-gated on Screener; the prior report therefore used the BSE-

filed investor decks and audited results, and this update does the same, supplemented by exchange filings and published coverage of the Q1 FY27 results.

Period	Source date	Coverage
Q3 FY24 → Q3 FY26 (9 quarterly decks)	Mar 2024 – Feb 2026	Carried forward from the Q4-2026 report
Q4 & FY26 (Mar 2026)	Deck dated 5 Jun 2026	Prior report's coverage boundary — carried forward
Q1 FY27 (Jun 2026)	Results 14 Aug 2026	Newly read in full ★

Sources used this quarter: Q1 FY27 results and published coverage (14 August 2026); the BSE filing of the board-approved ₹519 cr expansion (14 August 2026); the BSE encumbrance filing of 11 August 2026 covering the promoter-level pledge; India Ratings material on the company; and Screener.in financial statements. No management wording is quoted verbatim because no transcript is accessible.

2b. Latest update highlights — Q1 FY27 (results 14 Aug 2026) ★

- **Growth did not slow after the fill-up — it accelerated:** revenue **₹91.8 cr, up 37.9%**, pre-tax profit **₹22.4 cr, up 124.2%**, and net profit **₹16.6 cr, up 119.9%**. Operating margin widened again to 31%. This matters because the last report's main caveat was that the easy growth was largely behind.
- **The growth came from price, not just beds:** occupancy edged up to **82%** (from ~81%), but **ARPOB rose 17% to about ₹31,481 a day**. In prior years ARPOB grew in the mid single digits. A 17% jump means the hospital is treating a materially more complex, higher-value case mix rather than simply admitting more people.
- **The legacy block is full again — 98% occupancy.** This is the same condition that existed at Q3 FY24 and prompted the last expansion. The company has effectively run out of room a second time, which is the direct trigger for what follows.
- **A ₹519 cr expansion of ~591 beds was approved on 14 August 2026**, roughly doubling capacity, phased from **Q2 FY28 to Q2 FY30**: at Trichy, a **₹422 cr Super-Specialty Block (~400 beds)** and a **₹64 cr addition to the Mother & Child block (~100 beds)**; and — the strategically important part — a **₹33 cr, 91-bed Mother & Child hospital in Bengaluru**, due Q2 FY28. This is **KMC's first hospital outside Trichy**. Funding is stated as a combination of internal accruals and term debt.
- **A new governance item to watch, disclosed 11 August 2026:** the promoters of the 75% parent, Sri Kauvery Medical Care, have encumbered **46.93% of that parent's shares** — a pledge over 14.53% plus a non-disposal undertaking over 32.40% — creating an **indirect encumbrance over roughly 35.2% of KMC's own equity**. The security is for non-convertible debentures issued by **Prospera Healthcorp Private Limited**, with Catalyst Trusteeship as debenture trustee. This is borrowing at the parent group, not at KMC, but KMC shares are the collateral.

2c. Business mix & capacity (quick facts)

Domestic vs export: 100% domestic, and until now single-city — roughly **89% of in-patients come from in and around Trichy**. There is no export revenue; it is a hospital. By specialty, mother-and-child care is the largest slice at ~27%, followed by neurology ~18% and gastroenterology ~12%. By payer, about **70% is cash/self-pay**, 23% insurance and corporate, and 7% government schemes — an unusually cash-paying, low-receivable patient base.

The concentration is about to change. The Bengaluru mother-and-child hospital approved in August 2026 is the first facility outside Trichy in the listed company's history. At 91 beds it is small, but it directly addresses the single-city risk flagged in the last report.

Capacity utilisation: roughly **330 commissioned beds of ~450 licensed**, running at **82% occupancy** overall — but with the legacy multi-specialty block at about **98%**, i.e. genuinely full. Plain read: the headroom noted last quarter has largely been consumed, and the remaining licensed beds are not enough to sustain 30%-plus

growth. That is precisely why the board approved ~591 new beds. Once built, the bed base roughly doubles — but the first of it does not open until **Q2 FY28**, with the rest phased to **Q2 FY30**.

2d. Since our last report ★

Builds on the **Q4-2026 report** (dated 11 Jul 2026, covering the Q4 & FY26 deck of 5 Jun 2026), which concluded **durable earning trigger** → **YES** and **management consistent** → **YES**, with one open question: what drives the next leg of growth once the new block is full.

Prior read	What we said then	What the new quarter shows	Movement
Durable earning trigger	YES — the 200-bed block filling from 65% to 81% drove a 118% profit jump	Q1 FY27 revenue +37.9% and profit +119.9%, margin up to 31% — the engine did not stall after the fill-up completed	Confirmed
Management consistency	YES — warned FY25 profit would fall, it did; said the block would fill and lift profit, it has	Delivered another record quarter and then announced the next capex with full specifics and phasing	Confirmed
Open question: "the easy fill-up is mostly behind — what drives the next leg?"	Flagged as Unproven: "needs a new capex to reappear", none firmly announced in the decks read	Answered directly. A ₹519 cr, ~591-bed expansion board-approved 14 Aug 2026, phased Q2 FY28–Q2 FY30, with project-level costs and dates	Resolved
Risk: single-city, single-cluster (~89% of patients from around Trichy)	Flagged as lacking the geographic spread of larger chains	A 91-bed Bengaluru mother-and-child hospital approved for Q2 FY28 — the first facility outside Trichy. Small, but a genuine first step	Improving
Promise to watch: ARPOB gains as the growth lever	Graded Moderate — "steady, single-digit each year"	ARPOB up 17% YoY to ~₹31,481 — far above the historical single-digit pace, on a richer case mix	Improving
Capacity headroom	"Some headroom left before a fresh expansion would be needed" — ~330 of 450 licensed beds commissioned	Largely gone. Legacy block at 98% occupancy ; overall 82%. The expansion is now a necessity, not an option	Consumed
Risk: valuation (P/E 47)	"A lot of good news is already in the price"	P/E now 39.6x — still expensive, but de-rated as earnings outgrew the share price	Slightly better
New since last report	—	Promoter-level encumbrance: 46.93% of parent Sri Kauvery Medical Care's shares pledged/undertaken, an indirect encumbrance over ~35.2% of KMC's equity, securing NCDs of Prospera Healthcorp Pvt Ltd. Filed 11 Aug 2026	New watch-item
New since last report	—	A second J-curve is coming, by design. ₹519 cr funded partly by term debt, first beds opening Q2 FY28 — meaning new interest, depreciation and empty-bed costs	Predictable drag ahead

will hit before the revenue does,
exactly as in FY25

Verdict change this quarter: both verdicts unchanged, and the single open question from last time has been closed. The last report gave a "yes, but watch" on the trigger precisely because the beds had filled and no new project was announced; three months later the board has approved ~591 beds and ₹519 cr with dated, phased commissioning, plus a first step outside Trichy. Meanwhile the current business grew 37.9% on a 17% ARPOB gain rather than on occupancy, which shows a second lever working. Two things now need watching that did not exist before: the promoter-level encumbrance over roughly 35.2% of the listed equity, and the certainty that a fresh capex J-curve — interest and depreciation ahead of revenue — begins around FY28.

3. Earning Trigger thesis ★

The original trigger was a textbook capital-expenditure J-curve and it has fully played out: a 250-bed hospital at 100% occupancy could not grow, so a 200-bed mother-and-child block opened in January 2024, spent about eighteen months as a drag (empty beds, full fixed costs, new interest and depreciation — which is exactly why FY25 profit fell), then filled from 65% to 82% and, because hospital costs are largely fixed, each extra patient fell almost straight to profit. FY26 profit more than doubled. **What is new is that the story has a second act rather than an ending.** Q1 FY27 grew 37.9% with occupancy barely moving, because ARPOB rose 17% — the hospital is doing more complex, higher-value work in the same beds. And on 14 August 2026 the board approved **₹519 cr for ~591 beds**, roughly doubling capacity, phased from Q2 FY28 to Q2 FY30, including a first hospital outside Trichy. The honest implication of that approval is that **the same J-curve will repeat**: term debt, interest and depreciation will arrive before the new beds fill, so a period of compressed profit growth around FY28–FY29 should be expected rather than treated as a disappointment.

Trigger	What the record says	Evidence so far	Horizon	Strength	Change since last report
200-bed block ramp (first J-curve)	Flagged early fixed costs, then scale	Complete. Occupancy 65% → 82%; FY26 profit +118%; Q1 FY27 +120%	Done	Strong	Unchanged — fully delivered
ARPOB / richer case mix	Growth "across all specialties", better case mix	₹31,481/day, up 17% YoY — versus mid-single-digit growth in prior years	Ongoing	Strong	↑ Upgraded from Moderate — the pace has stepped up sharply
₹519 cr / ~591-bed expansion	Board-approved 14 Aug 2026; Trichy ₹486 cr (~500 beds), Bengaluru ₹33 cr (91 beds); phased Q2 FY28–Q2 FY30	Approved with project-level costs and dates. Justified by the legacy block being at 98% occupancy	FY28–FY30	Strong	↑ New — resolves the prior report's "Unproven / needs a new capex"
Geographic diversification (Bengaluru)	91-bed mother-and-child hospital, ₹33 cr, Q2 FY28	First facility outside Trichy in the listed company's history — but only ~15% of the new bed count	FY28	Moderate	↑ New — addresses the single-city risk, modestly
		Structural strength — low receivables and	Structural	Strong	Unchanged

Cash-paying patient base	~70% cash/self-pay, 23% insurance, 7% government	little dependence on slow-paying government schemes			
Capacity headroom (near term)	~330 of ~450 licensed beds commissioned	Largely exhausted — legacy block 98% full. Little room to grow volumes before Q2 FY28	Now–FY28	Constraint	↓ Weakening — the headroom noted last quarter is gone
Second J-curve (predictable drag)	₹519 cr funded by internal accruals <i>and term debt</i>	Interest, depreciation and empty-bed costs will land from ~FY28, before the beds fill — exactly the FY25 pattern	FY28–FY29	Known headwind	↓ New — the price of the next leg
Promoter encumbrance (risk)	46.93% of parent SKMCIL encumbered, securing third-party NCDs	Indirect encumbrance over ~35.2% of KMC's equity. Not KMC's debt, but KMC's shares are the collateral	Now	New risk	↓ New watch-item

4. Management Consistency scorecard

Read this as **limited** — transcripts are login-gated, so this grades BSE-filed investor decks, exchange filings and reported numbers rather than management's own recorded words.

Period	What was promised / said	What actually happened	Verdict
Q3 FY24	Legacy hospital 100% full; new 200-bed block just opened — growth to come from it	Correct starting point; the block opened and began taking patients	Delivered
FY25 (the dip)	Openly warned profit would be hit by additional fixed overheads, interest and depreciation from the new facility	Exactly as flagged — FY25 profit fell to ₹21 cr from ₹30 cr. They named the cause each quarter rather than spinning it	Delivered (honest)
FY26 (the payoff)	Implied the block would fill and margins recover	Occupancy to ~81%; FY26 revenue ₹306 cr (+32%), profit ₹47 cr (+118%)	Delivered
Q1 FY27	—	Revenue +37.9%, profit +119.9%, margin to 31%, ARPOB +17% — growth continued after the fill-up was complete	Delivered
Aug 2026	The prior report's gap: no announced project to drive the next leg	₹519 cr / ~591 beds approved with project-by-project costs, a Bengaluru entry, and phased commissioning dates Q2 FY28–Q2 FY30	Addressed
Aug 2026	Funding of the expansion	Stated plainly as internal accruals plus term debt — no vagueness, though it means leverage will rise from the current ~₹84 cr base	Disclosed
Aug 2026	—		

		Promoter-level encumbrance over 46.93% of the parent disclosed to BSE on 11 Aug 2026 with full detail — share counts, percentages, the debenture trustee and the issuing entity	Disclosed, but new risk
Ongoing	—	Transcripts remain login-gated; investors outside the calls rely on decks. No dividend despite three years of rising profit	Disclosure gap

Narrative drift: Still none, and the record has strengthened. For roughly three years the message has been one simple thread — build capacity, wear the near-term cost, fill it, repeat — and each stage has arrived when management said it would. The most creditable episode remains FY25: they told shareholders in advance that the new block would depress profit through extra fixed costs, interest and depreciation, it did exactly that, and they restated the reason every quarter instead of blaming something else. FY26 then delivered the payoff they had implied. This quarter adds a further mark in their favour: the one weakness identified in the last report — that the growth engine was running out of road with no replacement announced — was closed within three months by a board-approved, costed, dated expansion, including the first step outside Trichy that also answers the geographic-concentration criticism. The disclosure around the promoter encumbrance was likewise prompt and unusually granular, naming the exact share counts, the trustee and the issuing entity. Two caveats keep this from being flawless. Transcripts remain inaccessible, so this assessment still rests on decks rather than on management being questioned. And the encumbrance itself — 46.93% of the parent's shares committed against debentures issued by a separate company — is a decision taken above KMC's own management but bearing directly on KMC's minority shareholders, and it is the one development here that could change the picture if the parent's borrowing runs into difficulty.

5. Bottom line — two verdicts

Durable earning trigger? → YES vs last quarter: unchanged (the main caveat has been removed)

What moved it: the last report gave a qualified Yes because the beds had filled, the easy operating leverage was spent, and no replacement project had been announced. Both halves of that caveat have since been answered. First, Q1 FY27 showed the business can still grow strongly without more beds — revenue up 37.9% and profit up 120% on occupancy that barely moved, driven instead by ARPOB rising 17% to about ₹31,481 a day as the case mix moved toward complex, higher-value work. That is a second lever, and it had previously been growing only in single digits. Second, on 14 August 2026 the board approved **₹519 cr for roughly 591 beds** — a ₹422 cr super-speciality block and a ₹64 cr mother-and-child addition at Trichy, plus a 91-bed hospital in Bengaluru — phased from Q2 FY28 to Q2 FY30. That roughly doubles capacity and begins to fix the single-city dependence. The trigger is therefore live, evidenced, and now funded for a further five years. Two things a buyer should price in honestly: the near-term ceiling is real, with the legacy block at 98% occupancy and no new beds until FY28; and the expansion is part debt-funded, so the same J-curve that depressed FY25 profit will recur around FY28–FY29 when interest and depreciation land ahead of the patients.

Management consistent? → YES vs last quarter: unchanged (with one new caveat)

What moved it: nothing required correction, and the quarter added evidence in their favour. This management warned that FY25 profit would fall because of the new block's fixed costs, interest and depreciation; it fell exactly as described, and they repeated the reason each quarter rather than deflecting. They said the block would then fill and lift profit; occupancy went to 82% and FY26 profit rose 118%. This quarter they went further and closed the one gap the last report identified, approving a fully costed, dated, phased expansion within three months of that gap being noted — and choosing Bengaluru for part of it, which also addresses the geographic-concentration criticism. The funding structure was stated plainly rather than left vague. Over roughly three years the promised-versus-delivered record is clean, and a promoter group of practising doctors running a NABH-accredited hospital supports the

credibility. The verdict stays Yes, but with a caveat that did not exist last quarter and should be tracked: in August 2026 the promoters of the 75% parent encumbered **46.93% of that parent's shares** — an indirect encumbrance over roughly **35.2% of KMC's own equity** — as security for non-convertible debentures issued by a separate entity, Prospera Healthcorp. It was disclosed promptly and in detail, it is not KMC's own borrowing, and it says nothing about KMC's operating management. But it places a third party's credit risk on top of the listed company's shareholder register, and if the parent's debt were to come under strain that would matter here regardless of how well the hospital performs.

Prepared from public sources on 31 Aug 2026 — Q1 FY27 results and coverage (14 Aug 2026), the BSE filing of the board-approved ₹519 cr expansion (14 Aug 2026), the BSE encumbrance filing of 11 Aug 2026, India Ratings material, and Screener.in financial statements. The pre-FY27 record is carried forward from the Q4-2026 report of 11 Jul 2026, which was built from BSE-filed investor decks for Q3 FY24 through Q4 FY26. Concall transcript PDFs are login-gated on Screener and could not be read, so no management wording is quoted verbatim and the consistency read is correspondingly limited. **This is not investment advice. Do your own due diligence.**